THE MATERNITY METHOD

NCLEX CHEAT SHEET

Spontaneous Abortion, Incompetent Cervix & Intrauterine Fetal Demise

Maternal & Newborn › Pregnancy Loss

The Big Picture

Spontaneous abortion (miscarriage) = pregnancy loss **before 20 weeks**, most often from **chromosome problems**, not anything mom did. **Incompetent (insufficient) cervix** = the cervix **opens painlessly** in the 2nd trimester. **IUFD** (stillbirth) = baby dies in the uterus **at 20 weeks or later**. Nursing care is part **physical** and part **grief support**.

KNOW THE NUMBERS

< 20 wks	≥ 20 wks	12–14 wks	36–37 wks	< 25 mm
Loss = **spontaneous abortion**	Loss = **stillbirth / IUFD**	**Cerclage** placed	**Cerclage removed**	**Short cervix** on ultrasound

Types of Spontaneous Abortion

Type	What's happening	Cervix	Care
Threatened	Spotting, mild cramps; pregnancy **may continue**	**Closed**	Pelvic rest, watch bleeding, ultrasound
Inevitable	Bleeding, cramps, **membranes may rupture**; can't be stopped	**Open**	Prepare for loss/D&C
Incomplete	**Some** tissue passed; heavy bleeding	**Open**	**D&C** or misoprostol
Complete	**All** tissue passed; bleeding and cramps settle	**Closed**	Monitor; support
Missed	Baby died but **wasn't passed**; may have no symptoms	**Closed**	D&C, misoprostol, or wait
Septic	**Infection**: fever, foul discharge, belly pain	Varies	**IV antibiotics** + empty the uterus
Recurrent	**2 or more** losses in a row	Varies	Workup for causes

MEMORY HACK

Open or closed? The two I's are open

Inevitable	Incomplete	Threatened	Complete	Missed
open	**open**	closed	closed	closed

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Nursing Care: Miscarriage

- **Count/weigh pads**; save any tissue passed
- VS, Hgb/Hct; IV access if bleeding is heavy
- **Rh− → RhoGAM**
- Teach: no **sex or tampons** for about 2 weeks (or as ordered)
- Come back for **> 1 pad/hr**, **fever**, foul discharge, severe pain

Reassure her: miscarriage is **not** caused by exercise, sex, or stress.

Incompetent Cervix

- **Painless** cervical opening in the **2nd trimester**, **no contractions**
- **Risks:** prior cervical surgery (**LEEP**, cone biopsy), prior 2nd-trimester loss, uterine anomalies
- **Treat:** **cerclage** (stitch) at 12–14 wks, removed at 36–37 wks; **vaginal progesterone** for short cervix
- Report **contractions, leaking fluid, bleeding, pressure**: the stitch may need to come out

IUFD (Stillbirth)

- **Signs:** baby **stops moving**; no heart tones on Doppler → **confirm with ultrasound**
- **Causes:** placental problems, infection, cord accidents, diabetes, high BP, birth defects (often unknown)
- **Usually induced**; watch for **DIC** if the baby was retained for weeks (check clotting labs)
- **RhoGAM** if Rh−; offer **autopsy** and testing; discuss milk suppression

Grief Support: Do & Don't

Do

- Use the **baby's name**
- Offer to **see and hold** baby, as long as they want
- Offer **memory items**: photos, footprints, handprints, lock of hair, blanket
- Respect **cultural/religious** rituals
- Say "**I'm so sorry**"; sit with silence
- Screen for **depression**; offer support groups

Don't

- "You can have **another** baby"
- "It was **meant to be**" / "Everything happens for a reason"
- "At least it was **early**"
- **Rush** the family
- Hide the loss: mark the door/chart per unit policy so staff know

RED FLAGS · STOP AND CALL THE PROVIDER

Soaking > 1 pad/hr · **Fever** or foul discharge (septic) · **Dizziness, fast HR** · **Oozing**, bruising (DIC) · Leaking fluid or contractions with a **cerclage**

MUST-KNOW FOR THE NCLEX

- **Threatened = closed cervix**; inevitable and incomplete = **open**.
- Most early losses are from **chromosome problems**. Not her fault.
- **Incompetent cervix** = painless 2nd-trimester dilation → **cerclage**.
- **RhoGAM** after any loss if mom is Rh−.
- **IUFD:** use baby's name, offer holding and memories; watch for **DIC**.

QUICK CHECK · TEST YOURSELF

1. Spotting with a closed cervix at 9 weeks is a _____ abortion.
2. Painless cervical dilation at 18 weeks suggests _____ _____.
3. A cerclage is usually removed at ____–____ weeks.
4. A fetal demise retained for weeks increases the risk of _____.
5. A missed abortion has a _____ cervix.

Answers: 1. threatened · 2. incompetent cervix · 3. 36–37 · 4. DIC · 5. closed

For study use only. Always follow facility protocols, current guidelines, and provider orders.